

National Coverage Determination (NCD)

Blood Transfusions

110.7

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Tracking Information

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100-3

Manual Section Number

110.7

Manual Section Title

Blood Transfusions

Version Number

1

Effective Date of this Version

12/08/1994

Implementation Date

12/08/1994

Description Information

Benefit Category

Inpatient Hospital Services

Outpatient Hospital Services Incident to a Physician's Service

Please Note: This may not be an exhaustive list of all applicable Medicare benefit categories for this item or service.

Item/Service Description

Blood transfusions are used to restore blood volume after hemorrhage, to improve the oxygen carrying capacity of blood in severe anemia, and to combat shock in acute hemolytic anemia.

A. Definitions

1. Homologous Blood Transfusion

Homologous blood transfusion is the infusion of blood or blood components that have been collected from the general public.

2. Autologous Blood Transfusion

An autologous blood transfusion is the precollection and subsequent infusion of a patient's own blood.

3. Donor Directed Blood Transfusion

A donor directed blood transfusion is the infusion of blood or blood components that have been precollected from a specific individual(s) other than the patient and subsequently infused into the specific patient for whom the blood is designated. For example, patient B's brother predeposits his blood for use by patient B during upcoming surgery.

4. Perioperative Blood Salvage

Perioperative blood salvage is the collection and reinfusion of blood lost during and immediately after surgery.

Indications and Limitations of Coverage

B. Policy Governing Transfusions

For Medicare coverage purposes, it is important to distinguish between a transfusion itself and preoperative blood services; e.g., collection, processing, storage. Medically necessary transfusion of blood, regardless of the type, may generally be a covered service under both Part A and Part B of Medicare. Coverage does not make a distinction between the transfusion of homologous, autologous, or donor-directed blood. With respect to the coverage of the services associated with the preoperative collection, processing, and storage of autologous and donor-directed blood, the following policies apply.

1. Hospital Part A and B Coverage and Payment

Under §1862(a)(14) of the Act, non-physician services furnished to hospital patients are covered and paid for as hospital services. As provided in §1886 of the Act, under the prospective payment system (PPS), the diagnosis related group (DRG) payment to the hospital includes all covered blood and blood processing expenses, whether or not the blood is eventually used.

In a situation where the hospital operates its own blood collection activities, rather than using an independent blood supplier, the costs incurred to collect autologous or donor-directed blood are recorded in the whole blood and packed red blood cells cost center. Because the blood has been replaced, Medicare does not recognize a charge for the blood itself. Under PPS, the DRG payment is intended to pay for all covered blood and blood services, whether or not the blood is eventually used.

Under its provider agreement, a hospital is required to furnish or arrange for all covered services furnished to hospital patients. Medicare payment is made to the hospital, under PPS or cost reimbursement, for covered inpatient and outpatient services, and it is intended to reflect payment for all costs of furnishing those services.

2. Nonhospital Part B Coverage

Under Part B, to be eligible for separate coverage, a service must fit the definition of one of the services authorized by §1832 of the Act. These services are defined in 42 CFR 410.10 and do not include a separate category for a supplier's services associated with blood donation services, either autologous or donor-directed. That is, the collection, processing, and storage of blood for later transfusion into the beneficiary is not recognized as a separate service under Part B. Therefore, there is no avenue through which a blood supplier can receive direct payment under Part B for blood donation services.

C. Perioperative Blood Salvage

When the perioperative blood salvage process is used in surgery on a hospital patient, payment made to the hospital (under PPS or through cost reimbursement) for the procedure in which that process is used is intended to encompass payment for all costs relating to that process.

Transmittal Information

Transmittal Number

72

Revision History

11/1994 - Clarified coverage and payment policies. Effective date 12/08/1994. (TN 72)

Additional Information

Other Versions

Title	Version	Effective Between	
Blood Transfusions	1	12/08/1994 - N/A	You are here